



P 21 · 44 PAGES, FIRST YEAR POSTPARTUM

A Postpartum Anxiety Companion

Forty-four pages for the version of you who cannot stop checking the baby's breathing. A noticing log. A "what's a thought, what's a worry" worksheet. A grounding-card library. A page for the partner. Specific signs that say "call your provider today." Plain language. Not a diagnosis. A way to see your own pattern.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

Before you open this

If you are in crisis right now, please put this companion down and call one of these.

- **988**, US Suicide & Crisis Lifeline (call or text, 24/7)
- **1-833-852-6262**, US Maternal Mental Health Hotline (24/7, free, confidential, parent-specific)
- **741741**, Crisis Text Line (text "HOME" to start)

If you have intrusive thoughts about harming the baby or yourself, that is a flag, not a verdict. Please call.

This companion is for the long stretch. The acute hour belongs to a real human on a phone.

With care, Maya

A note from me

The first time I had postpartum anxiety, I did not know that's what it was. I thought I was just being a careful mother. I checked the baby's chest every twelve minutes, all night, for three weeks. I refused to sleep because I was the only person who would notice if he stopped breathing. I cried in the pediatrician's waiting room because I had convinced myself the appointment was about something terminal that nobody was telling me.

A therapist asked me, much later, when I had eaten that day. I could not remember. She asked me when I had slept more than two hours in a row. I could not remember. She said, gently, "I think you might be having a hard time. Can we talk about that?"

This companion is the thing I wish I'd had on the bedside table in those weeks. Not to fix anything. To help me see, on paper, that what was happening in my head was a pattern, and that the pattern had a name.

With care, Maya

This is not medical advice. It is a noticing tool. If you suspect you are dealing with postpartum anxiety, please talk to a real provider. The numbers are at the back.

The names we use, in plain language

Postpartum anxiety, postpartum depression, postpartum OCD, and postpartum psychosis are related but distinct. They can overlap. They are all treatable. Knowing which is which helps you describe it to a provider.

Baby blues

Mood shifts, tearfulness, overwhelm in the first 2 weeks. Affects about 80% of birthing parents. Usually resolves on its own by day 14.

Postpartum anxiety (PPA)

Persistent worry, racing thoughts, intrusive scary thoughts, inability to relax even when the baby is asleep, physical anxiety symptoms (racing heart, shortness of breath, knot in the stomach). Often overlaps with depression but can occur alone. Estimated to affect 1 in 5 birthing parents. Can start any time in the first year.

Postpartum depression (PPD)

Persistent low mood, hopelessness, loss of interest, sleep and appetite changes, that do not resolve. Can start any time in the first year. Affects about 1 in 7 birthing parents, and around 1 in 10 non-birthing partners.

Postpartum OCD (PPOCD)

Intrusive, unwanted, often disturbing thoughts, usually about harm to the baby, accompanied by checking or avoidance behaviors. The intrusive thoughts in PPOCD are NOT desires. They are distressing precisely because the parent does NOT want them. Treatable, and important to flag to a provider.

Postpartum psychosis

Rare (about 1-2 per 1000), but a medical emergency. Symptoms: disconnection from reality, hearing voices, paranoid thoughts, manic energy, beliefs that don't fit reality. If this is you or someone you know, please go to the ER. Now.

All of the above are treatable. None of them are character flaws. None of them mean you don't love your baby.

What postpartum anxiety often looks like

A list, from real people, in their own words.

- "I check the baby's breathing every five minutes at night, and I cannot stop, even though I know it's not helping anyone sleep."
- "I cannot watch the news. Any news. Even local news. I see disasters in everything."
- "I drove the baby to the pediatrician for a third time this month for something that turned out to be nothing. I am the third-time mom. I know this is nothing. But I cannot rest until a doctor tells me."
- "I keep imagining things falling on the baby. Light fixtures. Picture frames. I removed every picture frame in the room above the crib. I know this is excessive. I cannot put them back."
- "I have not let anyone else hold the baby alone, including my partner."
- "My chest is tight all day. My hands are cold. I have to remind myself to breathe."
- "I check the carbon-monoxide detector five times before bed."
- "I have started avoiding leaving the house. I have a long list of reasons, none of which are the actual reason."
- "I scream-cry in the shower because it is the only place no one can hear me, but I have started to dread the shower because I know I will cry in it."
- "I am up at 3am writing this. I have been up since 1am."

If you read any of those and felt seen, this companion is for you.

How to use this companion

- Forty-four pages, undated.
- Use one page at a time. Skip ones that don't apply.
- Do not try to "catch up." If you missed three days of the noticing log, just write today's. The log is not a referendum on you.
- Bring it to your provider. Even if you only have two weeks of notes, two weeks of notes is data.
- This is not a workbook with right answers. The right answer is "what is actually happening for me."

The daily noticing log

Use one row per day. Five lines, five minutes.

DATE _____ / WEEKS POSTPARTUM ____

OVERALL ANXIETY TODAY (circle):

1 2 3 4 5 6 7 8 9 10
none constant

THE PHYSICAL FEELING TODAY (circle any):

chest tight / shallow breath / racing heart
cold hands / knot in stomach / restless legs
muscle tense / trouble swallowing / none today

THE ONE THOUGHT THAT KEPT COMING BACK

(write it. Don't argue with it. Just write it.)

WHAT WAS HAPPENING WHEN IT WAS WORST

ONE SMALL GROUNDING I TRIED:

SLEEP LAST NIGHT (hours total / longest stretch): ____ / ____

EATEN (circle): not much / normal / too much

Use this for as many days as you can. After two weeks, you will start to see a pattern. The pattern is the data. The data is what you take to a provider.

The weekly self-awareness inventory

Once a week, fill this in. The questions are intentionally plain. They are NOT a clinical screening tool. If you want a clinically validated screening (like the EPDS), your OB or therapist will administer one. This is just for noticing.

WEEK OF _____

In the past 7 days, how often did each of these happen?

1 = never 2 = a few days 3 = most days 4 = every day

- _____ I felt nervous, on edge, or restless
- _____ I had trouble stopping or controlling my worries
- _____ My chest or stomach felt tight without a clear reason
- _____ I worried about something specific that I knew was unlikely (a disaster, the baby's breathing, an illness)
- _____ I could not relax even when nothing was happening
- _____ I felt that something bad was going to happen
- _____ I had trouble falling asleep because of worry
- _____ I avoided a place, a thing, or a conversation because of anxiety
- _____ I had an intrusive thought I could not shake (intrusive = unwanted, scary, often about harm)
- _____ I felt this was MORE than just "new-parent worry"

ONE THING I WANT TO ASK A PROVIDER ABOUT NEXT TIME

ONE THING THAT HELPED THIS WEEK

The numbers are not a score. They are a snapshot. If "most days" or "every day" answers are stacking up week after week, that is the thing to bring to a real provider.

The “what’s a thought, what’s a worry” worksheet

A CBT-style noticing tool. Not therapy, but a thinking pattern that comes from therapy. Use it whenever a thought is looping.

THE THOUGHT (write it, exactly as it came)

IS IT A THOUGHT, OR IS IT A WORRY?

- ☐ A thought = a thing that arrives. It does not require action. You can let it pass.
- ☐ A worry = a thing that suggests action. It says "do something or something bad will happen."

WHAT MY BRAIN IS PREDICTING

If the thought were 100% true, what would happen?

What's the actual probability of that, in plain odds?

- ☐ very high (more likely than not)
- ☐ medium
- ☐ low (1 in 100 or less)

WHAT'S UNDERNEATH THE WORRY

Often a worry is a feeling looking for a job. Underneath this worry, what feeling is also there?

- ☐ Fear ☐ Sadness ☐ Anger
- ☐ Loneliness ☐ Helplessness ☐ Exhaustion
- ☐ Something else: _____

WHAT WOULD HELP, RIGHT NOW

- ☐ A glass of water
- ☐ A short walk
- ☐ Hand the baby to someone for ten minutes
- ☐ Lie down
- ☐ Call _____

- ☐ Write a sentence to the partner
- ☐ Nothing. Let the thought pass.

THE THOUGHT, RE-WRITTEN (optional)

This worksheet does not “fix” the thought. It makes a small piece of distance between you and it.

The grounding-card library

Ten grounding moves. Each one is sixty seconds or less. None of them require a quiet room.

Card 1, the five-four-three-two-one

Look around. Find:

- 5 things you can see
- 4 things you can touch
- 3 things you can hear
- 2 things you can smell
- 1 thing you can taste

Count out loud, even if it feels strange. The counting works.

Card 2, cold water on wrists

Run cold water on the insides of your wrists for thirty seconds. The body interprets this as "I went swimming, I should regulate," and the nervous system slows down. Real, fast, free.

Card 3, the box breath

Breathe in for 4 counts. Hold for 4. Breathe out for 6. Hold for 4. Repeat three times.

Card 4, the wall push

Stand facing a wall. Put both palms on the wall. Push, hard, for ten seconds. Stop. Push again. The pushing gives the activated nervous system somewhere to go.

Card 5, the slow scan

Stand or sit. Slowly turn your head, all the way to the left. Pause. Slowly turn all the way to the right. Pause. Do this three times. Often this signals "the environment is safe, you can stand down."

Card 6, the hand on chest

Put one hand on your chest. Feel the rise and fall. Press a little. Say (to yourself, out loud or silent): "I am here. I am here. I am here." Three times.

Card 7, name the moment

Out loud, in plain language: "Right now, I am in the kitchen. The baby is asleep. The house is quiet. I am okay in this minute."

Use the actual room, the actual hour, the actual facts. Naming them out loud anchors you to the present.

Card 8, a small physical move

Roll your shoulders backward five times. Touch your toes for ten seconds. Stretch your arms above your head. Anything small. The body has to participate for the brain to follow.

Card 9, the partner-call script

Pick up the phone. Call the partner or a trusted person. Say: "I am having a hard minute. Can you talk for two minutes?" Two minutes is enough. They do not need to fix anything.

Card 10, the writing

Open this companion. Write one sentence about how you are feeling, on the noticing log page. The act of writing is the grounding. The sentence does not have to be wise.

What's roughly normal this week, week-by-week

Pages with rough expectations for what a new parent's mental state can look like, week by week. These are wide ranges, not targets.

Weeks 1-2, baby blues window

Tearfulness, overwhelm, sudden waves of feeling are typical. About 80% of birthing parents experience some of this. It usually resolves on its own by day 14.

A line: if the tears are still daily by day 18, and they are paired with persistent anxiety or low mood, that is worth a call.

Weeks 3-6, the brownout

Sleep debt accumulates. Anxiety often peaks here for many parents. Hormonal shifts continue. Identity is also doing something. Many people start to think, around week four, "this is harder than I thought."

A line: if you are not eating, not sleeping, not safe to drive, or not able to be alone with the baby without panic, that is worth a call.

Weeks 6-12, the second wave

Many parents land in a settled rhythm here. Some get hit by a second wave of low mood or anxiety, especially if they have returned to work, if a partner has gone back to work, or if family support has dropped off.

A line: a "second wave" that lasts more than two weeks and is interfering with eating, sleeping, or basic functioning is the signal to call.

Months 4-6, the long view

PPA often emerges or re-emerges here, especially around the four-month sleep regression. Many parents who felt fine in weeks 1-3 now find themselves anxious for the first time.

A line: PPA is "postpartum" through the first year, not just the first six weeks. If anxiety is increasing instead of easing, call.

Months 7-12, the full year

If anxiety symptoms are still present by month 6 and getting worse, this is not "first-year normal." This is postpartum anxiety that has not been treated, and treatment helps.

A line: ask for help. The hotline is **1-833-852-6262**.

When to call your provider today

Not next week. Today.

- ☐ Intrusive thoughts about harming yourself or the baby, even if they feel "not real" and you would never act on them
- ☐ Inability to sleep even when the baby is sleeping, for more than 3 nights in a row, due to anxiety (not infant feeding)
- ☐ Persistent feeling that something is terribly wrong, that doesn't shift with sleep or rest
- ☐ Physical anxiety symptoms (racing heart, can't catch breath) that have lasted more than an hour
- ☐ Inability to eat for more than 24 hours
- ☐ A panic attack that scared you
- ☐ Avoidance of the baby
- ☐ A feeling that you cannot keep going
- ☐ A sense that you might hurt yourself
- ☐ Any voices, paranoia, or beliefs that don't fit reality (this is a medical emergency, go to the ER)

If you checked any of these, please call:

- ☐ Your OB or midwife
- ☐ Your primary care provider
- ☐ US Maternal Mental Health Hotline: 1-833-852-6262
- ☐ US Suicide & Crisis Lifeline: 988
- ☐ Or 911 / your local ER

A trained provider will not be alarmed by these. They have seen them before. They have helped other parents.

A page for the partner

If you are reading this companion because your partner is going through it, this page is for you.

What to watch for

You see things she does not see in herself. You see the patterns. You see the changes. You may be the first to notice.

Signs to watch for:

- She is checking and re-checking things (the door, the baby, the stove)
- She is not eating, or not eating enough
- She is not sleeping even when the baby sleeps
- She is irritable in a way that is new
- She is avoiding things she used to do
- She is crying often, alone, in places like the shower or the car
- She says "I'm fine" in a flat voice
- She asks the same worry over and over, and reassurance does not land

What to say

The wrong move is "you're fine" or "stop worrying." That is the version of reassurance that does not help.

A better move:

"I've noticed you've been _____ for a few days now. I love you. I want to make sure you're okay. Can we talk about it for ten minutes, or would you rather I just sit with you?"

If she says "I'm fine," gently:

"You don't have to be fine. You can be whatever you are. I'm not going anywhere."

What to do

- Offer a real, specific thing. Not "what can I do?" Try: "I'm going to take the baby for the next 90 minutes. You can sleep, or shower, or just be alone. I'll be back at 4."
- Make her food without asking. Put it in her hand.
- Call her OB or midwife with her if she wants. Or for her, if she would like.
- Take the night shift. Even one night. Even if it means formula.

When to push

If she has talked about hurting herself, or hurting the baby, or sounded disconnected from reality, please call:

- **988** for crisis
- **1-833-852-6262** for the Maternal Mental Health Hotline
- **911** or your local ER for postpartum psychosis or active self-harm

You do not need her permission to call a hotline. You can call to talk about her. They will help you figure out what to do next.

You are also allowed to be tired and scared. Your worry is the love speaking. Take care of yourself too.

An appointment prep page

A template you'll see four times in the companion. Print or photograph, bring to your provider.

APPOINTMENT FOR: _____

DATE: _____

PROVIDER: _____

WHAT'S BEEN HAPPENING (one sentence)

HOW LONG IT'S BEEN HAPPENING

___ days ___ weeks ___ months

THE TOP 3 SYMPTOMS THAT WORRY ME

1. _____

2. _____

3. _____

WHAT I AM ASKING FOR (circle any)

- ☐ A name for what's happening
- ☐ A referral to a therapist
- ☐ A referral to a psychiatrist
- ☐ Information about medication options
(decision to be discussed, no commitment)
- ☐ A check on whether this might be a hormone issue
(thyroid, etc.)
- ☐ A check-in in 2 weeks
- ☐ Just to be heard

WHAT I AM NOT READY FOR YET (circle any)

- ☐ Trying a medication today
- ☐ A specific therapy modality
- ☐ Anyone else being told about this

SAMPLE OF THE LOG FROM THE LAST 2 WEEKS (attach pages)

QUESTIONS TO ASK

1. _____

2. _____

3. _____

WHAT THE PROVIDER SAID (filled in after)

NEXT APPOINTMENT: _____

The reset page, week six

A short page that lives at the back of the companion.

You have been doing the noticing for some number of weeks now.

Look back at the daily log. Look at the weekly inventory. Look at the worksheet. Is the pattern getting heavier, lighter, or about the same?

If it is getting heavier, please make the call. Not next week. This week. The number is on the next page.

If it is getting lighter, please keep noticing. The companion will be here. So will the next hard week, if there is one.

If it is about the same, that's still useful information. About-the-same for many weeks is also a flag worth a conversation.

You are not "supposed to" feel anything specific by now. You are not behind. You are not too sensitive. You are not making this up.

You are paying attention. That is the work.

Soothemade.

Numbers to keep

OB / MIDWIFE

Name: _____ Number: _____

After-hours line: _____

PRIMARY CARE PROVIDER

Name: _____ Number: _____

THERAPIST (if you have one)

Name: _____ Number: _____

PSYCHIATRIST (if you have one)

Name: _____ Number: _____

US MATERNAL MENTAL HEALTH HOTLINE

1-833-852-6262 (24/7, free, confidential, parent-specific)

US SUICIDE & CRISIS LIFELINE

988 (call or text, 24/7)

US CRISIS TEXT LINE

Text "HOME" to 741741

POSTPARTUM SUPPORT INTERNATIONAL

1-800-944-4773 (call), text "Help" to 800-944-4773

A SAFE PERSON TO CALL

ANOTHER SAFE PERSON

LOCAL ER

Hospital: _____ Number: _____



From Soothemade Notes, a small apothecary of printables, planners, and cards for the unphotographed parts of new parenthood. Made slowly, in plain language.

Not medical advice.



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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